

Additional clinical information — MRI lumbar spine + sacrum

For the protocolling / reporting radiologist. Patient: Yujie _____ DOB: ____ / ____ / ____

Date: 18 Aug 2026

Referral states: *"Left posterior thigh pain radiating to left heel to exclude radiculopathy. Urinary incontinence occasionally to exclude underlying cause of overactive bladder."*

Clinical concern: occult spinal dysraphism / tethered cord, or sacral root lesion (LEFT).

Please report specifically on the **conus level and filum terminale**, and on the **sacral canal, foramina and roots to the coccyx** — in addition to the L4–S1 discs.

History not on the referral

- **Pain distribution:** arises at the **LEFT side of the vaginal introitus** and runs continuously down the posterior thigh to the heel (perineal → S2 → S1). ~12 months, progressively more constant and severe. Perineal component spontaneous, electric-shock quality, relieved by pressure, worse recumbent.
- **Lifelong** severe constipation since early childhood (retention; 4–6 days between BMs, no urge); recent change to looser stools.
- **Lifelong** urinary incontinence (urodynamics previously performed).
- Reduced perineal / genital sensation.
- **Lifelong colder, smaller LEFT leg; unilateral LEFT pes cavus with claw toes;** left calf thinner; left toe-walk / heel-walk weaker.
- **Cutaneous marker over the sacrum.**
- Pelvic floor dysfunction on physiotherapy assessment. No SSRIs / OCP. No prior spinal surgery.

Specific questions

1. **Conus medullaris level; filum terminale** — low-lying conus? thickened (>2 mm) or fatty filum? filar / conus lipoma? dermal sinus tract? (tethered cord — note adult tethering can occur with a normal-level conus)
2. **Sacral canal, foramina and roots, S1–S5 to coccyx** — perineural (Tarlov) cysts? sacral dysgenesis / spina bifida occulta? any lesion at the **LEFT S1–S4 roots**, sacral plexus or greater sciatic notch that could account for a combined perineal + posterior-leg distribution?
3. L4–S1 disc / foraminal pathology correlating with left S1 symptoms.
4. Cauda equina — any compromise.

Sequences at the radiologist's discretion; fat-sensitive sagittal imaging of the filum and axial coverage through the sacrum would be appreciated. Photographs of the foot asymmetry and sacral marker are available from the patient on request.

Practical

- Pain is worse lying flat — knee bolster and/or a short break mid-study may help her keep still.
- Patient requests a copy of the images (portal link or disc) and her own copy of the report, in addition to the referring GP.

Thank you. Prepared by the patient's partner from her history; the referring GP is being asked to forward an amended request.